INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Palbociclib with endocrine therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Regimen source
NCCP 00414
Version 5a

Oral treatment with a planned rest week

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Palbociclib slows cell-cycle progression while the hormone partner reduces oestrogen signalling. The aim in this advanced-disease pathway is cancer control, with treatment and response reviewed over time.

CDK4/6 INHIBITOR

Palbociclib

Blocks cell-cycle signals that help cancer cells divide

CDK4/6 drives cell-cycle entry → Palbociclib blocks CDK4/6 → Cell division slows

Hormone signalling can activate cyclin-D/CDK4/6. Palbociclib blocks this control point, helping keep susceptible cells from progressing through the cell cycle. Combining it with endocrine therapy acts on both the hormonal signal and downstream cell division.

The key early issue is a falling white-cell count, which may cause no symptoms. Keep blood-test appointments and report infection symptoms or new cough/breathlessness promptly.

EITHER AN AROMATASE INHIBITOR OR FULVESTRANT

Your endocrine partner

Reduces oestrogen signalling to hormone-sensitive cells

Oestrogen can stimulate cancer growth → aromatase inhibitor (AI) lowers oestrogen; fulvestrant targets its receptor → Hormone-driven growth is reduced

An aromatase inhibitor lowers oestrogen production; fulvestrant acts at the oestrogen receptor. These are alternative partners here, not an instruction to take both. The partner's dose, monitoring and injection or tablet schedule must be confirmed.

Discuss joint/hot-flush and bone symptoms with an AI, or injection-site/bleeding symptoms with fulvestrant. Pre/perimenopausal women require ovarian suppression in the prescribed combination.

Your treatment aim

Who this treatment is for

HR-positive/HER2-negative locally advanced/metastatic disease with an AI, or with fulvestrant after previous endocrine therapy as defined in NCCP. Pre/perimenopausal endocrine treatment requires ovarian suppression; the exact partner and its risks must be confirmed.

NCCP 00414 v5a · official regimen

Schedule and duration

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required. Repeat while beneficial and tolerated. The planned week off concerns palbociclib, not automatically the endocrine partner.

NCCP 00414 v5a · official regimen

What the studies tell us

Palbociclib with hormone therapy can delay cancer growth. Studies used different hormone partners and different prior-treatment groups. The final overall trial results did not establish a clear survival improvement with palbociclib; later exploratory follow-up needs to be interpreted separately.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

PALOMA-2 primary report, Finn et al 2016 · PALOMA-2 final OS, Slamon et al 2024 · PALOMA-3 protocol-specified final OS, Turner et al 2018 · PALOMA-3 exploratory extended follow-up, Cristofanilli et al 2022

PALOMA-2 and PALOMA-3: keep endocrine partner and analysis separate

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the treatment team promptly if you feel feverish or unwell, have shaking chills or infection symptoms, or develop new cough or breathlessness. Follow the local emergency temperature instructions; do not wait for a planned blood test or for the 38.5°C value used in one clinician dose-adjustment table. Severe breathing difficulty needs emergency care.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

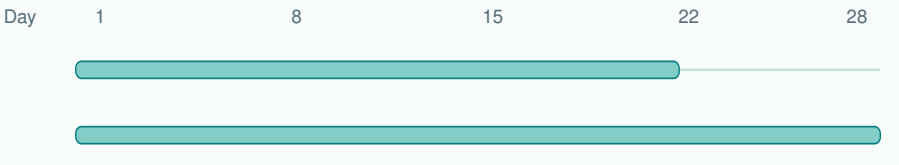
The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00414 v5a · official regimen · EMA Ibrance SmPC / patient leaflet; product information updated 7 March 2025, sections 4.1–4.8 and 5.1

THE WHOLE TREATMENT JOURNEY

Palbociclib with endocrine therapy: treatment and checks

Palbociclib with endocrine therapy

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

HR-positive, HER2-negative locally advanced or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycles 1–2	Cycles 3–6	After cycle 6	Hold / change / completion
Treatment NCCP 00414 · SMPC	Confirm AI or fulvestrant and any ovarian suppression	Palbociclib Days 1–21; rest Days 22–28; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	Continue while beneficial and tolerated	Confirm each medicine's plan separately
Blood count NCCP 00414 · SMPC	Baseline full blood count (FBC)	Day 1 and Day 15 in both cycles	Day 1; extra tests as indicated	Only after at most grade 1–2 neutropenia: may reduce to every three months before a cycle	Low counts, fever or infection require extra review
Renal / liver NCCP 00414 · SMPC	Baseline profiles	Before each cycle	Before each cycle	Before each cycle	Review changes before restarting

Care / check	Before starting	Cycles 1–2	Cycles 3–6	After cycle 6	Hold / change / completion
electrocardiogram (ECG) / echo NCCP 00414 · SMPC	Only if clinically indicated	No routine palbociclib-specific ECG date	Investigate symptoms/ risk when indicated	Do not copy ribociclib ECG timing	Follow any identified problem
Endocrine / bone / injections NCCP 00414 · SMPC	Confirm partner and bone-risk plan	Fulvestrant has its own loading/ injection schedule if used	AI daily or fulvestrant as prescribed	Ovarian suppression and bone review as indicated	Review partner continuation separately
Cancer response NCCP 00414 · SMPC	Baseline disease assessment	Review symptoms and tolerability	Imaging according to consultant plan	Continue response and symptom review	Progression prompts a new treatment decision

What each check is for

1 Before the first dose

Before starting

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

2 Early toxicity checks

Cycles 1–2

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

3 Benefit, tolerability and adherence

After cycle 6

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

4 Completion or treatment change

Hold / change / completion

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

FBC, renal and liver profile. Confirm receptor status, exact endocrine partner, previous endocrine therapy, menopause/ovarian suppression, pregnancy precautions and interacting medicines.

During treatment

FBC on Day 1 and Day 15 of cycles 1–2; Day 1 from cycle 3 and extra checks as indicated. Only after at most grade 1–2 neutropenia throughout the first six cycles may FBC monitoring reduce to every three months, before the start of a cycle and as indicated. Renal and liver profile before each cycle; review fever/infection, mouth/bowel symptoms, fatigue, pulmonary symptoms and adherence. No routine palbociclib-specific Day-14 ECG or echo timetable; investigate when clinically indicated.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Palbociclib	 **Days 1–21** Daily prescribed dose on Days 1–21
Palbociclib rest week	 **Days 22–28** No palbociclib; next cycle only after team confirmation
Endocrine partner	 **Days 1–28** AI every day OR fulvestrant at its scheduled injection visits
Blood-count changes	 **Days 1–28** May occur without symptoms. full blood count (FBC) Day 1 and Day 15 of first 2 cycles; extra tests if low.
Symptoms vary between people	 **Days 1–28** Mouth soreness, nausea or fatigue can occur; no guaranteed day-by-day prediction.

Treatment and rest: Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.

Tests and appointments: Use the monitoring table for baseline and later checks. A rest week does not remove infection or other risks.

> **Why these bands stay broad:** Treatment days are specified by the regimen. Symptom timing varies; the full-cycle symptom bands show that an effect can occur at any point, not a measured probability or severity. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00414 v5a · official regimen ↗ · EMA Ibrance SmPC / patient leaflet; product information updated 7 March 2025, sections 4.1–4.8 and 5.1 ↗

There are no guaranteed "safe days". Contact the treatment team promptly if you feel feverish or unwell, have shaking chills or infection symptoms, or develop new cough or breathlessness. Follow the local emergency temperature instructions; do not wait for a planned blood test or for the 38.5°C value used in one clinician dose-adjustment table. Severe breathing difficulty needs emergency care.

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Nausea support

Palbociclib is oral minimal–low emetic risk. Premedication is not usually required; agree rescue treatment if symptoms occur.

Blood counts and infection

Attend early and pre-cycle blood tests. Dose holds and reductions are clinician decisions. Growth-factor injections are not a routine automatic prescription for every patient.

Endocrine-partner support

Agree help for hot flushes, joints, vaginal or sexual symptoms, sleep and bone health. Fulvestrant injection discomfort and any ovarian-suppression injections have separate support plans.

Formulation and interactions

Check whether the pack contains capsules or tablets and follow the food instructions for that product. Check new medicines, supplements and grapefruit interactions.

Symptoms between visits

Report mouth ulcers, persistent bowel symptoms, worsening fatigue, fever or new breathing symptoms promptly.

When the plan changes

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

Practical support

Nausea support

Blood counts and infection

Endocrine-partner support

Formulation and interactions

Symptoms between visits

Taking tablets / attending treatment

Practical treatment details

Your endocrine partner has a separate timetable

If fulvestrant is the chosen partner, the usual schedule is 500 mg by intramuscular injection on Days 1, 15 and 29, then once monthly; confirm the prescribed dose and appointment dates.

Supply and appointments

Request the next supply before tablets run out.

This oral treatment does not itself require a central line or infusion pump.

Questions to ask

Which checks happen before and during treatment?

FBC, renal and liver profile. Confirm receptor status, exact endocrine partner, previous endocrine therapy, menopause/ovarian suppression, pregnancy precautions and interacting medicines. FBC on Day 1 and Day 15 of cycles 1–2; Day 1 from cycle 3 and extra checks as indicated. Only after at most grade 1–2 neutropenia throughout the first six cycles may FBC monitoring reduce to every three months, before the start of a cycle and as indicated. Renal and liver profile before each cycle; review fever/infection, mouth/bowel symptoms, fatigue, pulmonary symptoms and adherence. No routine palbociclib-specific Day-14 ECG or echo timetable; investigate when clinically indicated.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Check whether the pharmacy supplied capsules or tablets: capsules are taken with food; tablets can be taken with or without food. Swallow whole at a consistent time. Avoid grapefruit. If you miss or vomit a dose, do not take an extra dose; take the next scheduled dose. Do not restart after a clinical hold until the treating team confirms. A hold may affect one medicine rather than the whole combination; confirm each component.

What happens after treatment changes or ends?

Repeat while beneficial and tolerated. The planned week off concerns palbociclib, not automatically the endocrine partner. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Palbociclib with endocrine therapy · supportive-care plan

Active medicines: Palbociclib + Your endocrine partner

Emetogenic risk: Oral minimal–low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Palbociclib is oral minimal–low emetic risk. Premedication is not usually required; agree rescue treatment if symptoms occur.
- Attend early and pre-cycle blood tests. Dose holds and reductions are clinician decisions. Growth-factor injections are not a routine automatic prescription for every patient.
- Agree help for hot flushes, joints, vaginal or sexual symptoms, sleep and bone health. Fulvestrant injection discomfort and any ovarian-suppression injections have separate support plans.
- Check whether the pack contains capsules or tablets and follow the food instructions for that product. Check new medicines, supplements and grapefruit interactions.
- Report mouth ulcers, persistent bowel symptoms, worsening fatigue, fever or new breathing symptoms promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Palbociclib with endocrine therapy

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required.

Oral CDK4/6 inhibitor + hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood counts

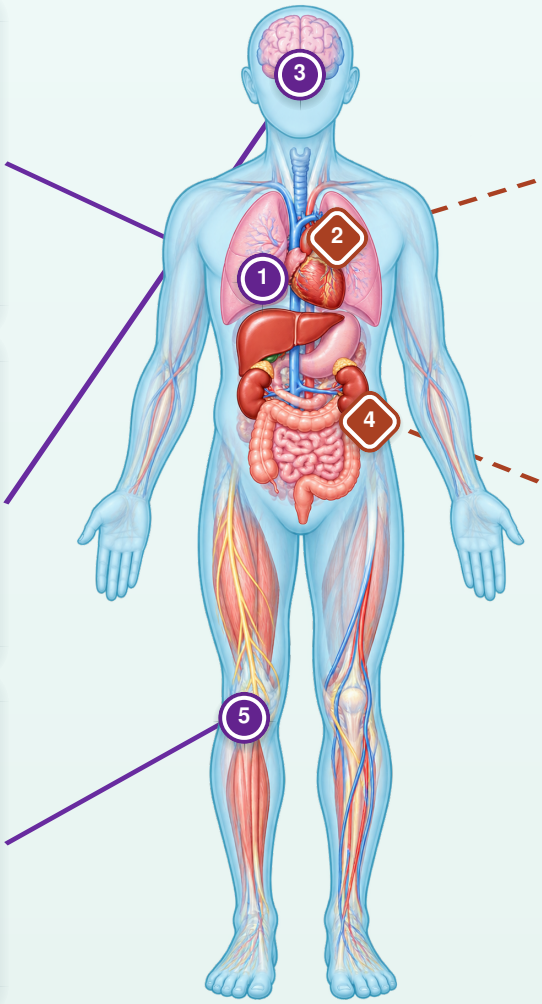
Palbociclib

Counts can fall without symptoms. Keep full blood count (FBC) appointments and report infection symptoms.

2 Lungs

Palbociclib

New cough or breathlessness needs prompt assessment for infection or uncommon lung inflammation.



3 Mouth

Palbociclib

Report painful sores or difficulty eating/drinking.

4 Energy

Palbociclib

Report fatigue that limits activity or is worsening; blood counts and other causes may need review.

5 Joint / bone symptoms

Endocrine partner

aromatase inhibitor (AI) and ovarian suppression may affect joints and bone; agree the partner-specific bone plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the treatment team promptly if you feel feverish or unwell, have shaking chills or infection symptoms, or develop new cough or breathlessness. Follow the local emergency temperature instructions; do not wait for a planned blood test or for the 38.5°C value used in one clinician dose-adjustment table. Severe breathing difficulty needs emergency care. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00414 v5a · official regimen ↗**
   https://healthservice.hse.ie/documents/6457/414_Palbociclib.pdf
2. **EMA Ibrance SmPC / patient leaflet; product information updated 7 March 2025, sections 4.1–4.8 and 5.1 ↗**
   https://www.ema.europa.eu/en/documents/product-information/ibrance-epar-product-information_en.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
6. **PALOMA-2 primary report, Finn et al 2016**
   https://www.nejm.org/doi/full/10.1056/NEJMoa1607303
7. **PALOMA-2 final OS, Slamon et al 2024**
   https://pubmed.ncbi.nlm.nih.gov/38252901/
8. **PALOMA-3 protocol-specified final OS, Turner et al 2018**
   https://pubmed.ncbi.nlm.nih.gov/30345905/
9. **PALOMA-3 exploratory extended follow-up, Cristofanilli et al 2022**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC9662922/
10. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf